

History and Physical Examination

****Patient Demographics:**** - Name: John Doe - Date of Birth: January 1, 1980 (Age: 46) - Gender: Male - Medical Record Number: 123456789 - Date of Examination: January 26, 2026 - Attending Physician: Dr. Jane Smith

****Chief Complaint:**** Chest pain for 2 hours.

****History of Present Illness:**** Mr. Doe is a 46-year-old male who presents to the emergency department with sudden onset of substernal chest pain that began 2 hours ago while at rest. The pain is described as sharp, 8/10 in intensity, radiating to the left arm and jaw. Associated symptoms include shortness of breath, nausea, and diaphoresis. He denies any recent trauma, fever, or cough. He has a history of hypertension and smoking. He took aspirin 325 mg at home without relief.

****Past Medical History:**** - Hypertension (diagnosed 5 years ago, on lisinopril 10 mg daily) - No known allergies - No previous surgeries - No hospitalizations

****Social History:**** - Smoker: 1 pack per day for 20 years - Alcohol: Occasional - Occupation: Office worker - No illicit drug use

****Family History:**** - Father: Died of myocardial infarction at age 60 - Mother: Alive, hypertension - No siblings

****Review of Systems:**** - Constitutional: No fever, chills, or weight loss - Cardiovascular: Chest pain as above, no palpitations - Respiratory: Shortness of breath, no cough or hemoptysis - Gastrointestinal: Nausea, no vomiting or diarrhea - Genitourinary: No dysuria or frequency - Neurological: No headache, dizziness, or weakness - Musculoskeletal: No joint pain or swelling

Physical Examination:

****Vital Signs:**** - Blood Pressure: 160/95 mmHg - Heart Rate: 95 bpm - Respiratory Rate: 20/min - Temperature: 98.6°F - Oxygen Saturation: 96% on room air

****General:**** Well-developed male in moderate distress due to chest pain.

****HEENT:**** Normocephalic, atraumatic. Pupils equal, round, reactive to light and accommodation. No nasal discharge. Oropharynx clear.

****Neck:**** No jugular venous distention. Carotid pulses 2+ bilaterally, no bruits.

****Cardiovascular:**** Regular rate and rhythm. No murmurs, rubs, or gallops. Point of maximal impulse normal.

****Respiratory:**** Lungs clear to auscultation bilaterally. No wheezes, rales, or rhonchi.

****Abdomen:**** Soft, non-tender, non-distended. Normoactive bowel sounds. No organomegaly.

****Extremities:**** No clubbing, cyanosis, or edema. Peripheral pulses 2+.

****Neurological:**** Alert and oriented x3. Cranial nerves II-XII intact. Strength 5/5 in all extremities. Sensation intact.

****Assessment:**** 46-year-old male with acute chest pain, likely acute coronary syndrome given risk factors and presentation.

****Plan:**** 1. Obtain ECG, cardiac enzymes (troponin), and chest X-ray. 2. Start aspirin 325 mg, nitroglycerin sublingual, oxygen, and morphine for pain. 3. Admit to cardiac care unit for monitoring. 4. Consult cardiology for possible cardiac catheterization. 5. Lifestyle modifications: Smoking cessation, blood pressure control.